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Identify the ROI: How hospitals can achieve sustainable innovation in an era of uncertainty

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Many leaders of America's health systems are walking a fiscal tightrope with their organization's long-term viability balanced on their shoulders. Margins are thin and the stakes are high. Healthcare costs have been on the rise for decades. As the national healthcare spend approaches 20 percent of gross domestic product, there's a growing sense that the results of an unaddressed cost crisis could soon be calamitous.

Enter value-based care. These new models of reimbursement are meant to incentivize providers to deliver superior care by linking patient outcomes to payment. However, while many experts argue healthcare's traditional fee-for-service payment models have contributed to the nation's ballooning healthcare cost crisis, transitioning to value-based models brings its own challenges for many organizations.

Reimbursements are down and hospitals have little room for error when it comes to operational efficiency. In the middle of last decade, the average operating margin fell by nearly 39 percent, from 4.15 percent in 2015 to 2.56 percent in 2017, according to a Navigant analysis. More recent research from Kaufman Hall suggests the downward trend persisted through the end of the decade with hospital margins dropping by more than 21 percent between 2018 and 2019.

Despite these considerable financial constraints facing the nation's providers, innovation in medicine and medical science has not slowed. Technology companies and medical equipment manufacturers continue to release sophisticated systems and tools at a steady clip. Every leader wants to outfit their facilities with the best tools and technologies available, but budgets restrict their purchasing power. In this environment, health system leaders must ensure they're getting their dollars' worth from investments in technology and equipment— with the ultimate return on investment being better patient outcomes.

"There's a lot of financial pressure on health systems, and this pressure often dictates what new technologies providers can afford to buy," said Julie Viola, connected solutions director with Philips, during an interview with *Becker's Hospital Review*. "Providers are trying to find the balance between maintaining their existing systems, bringing costs down and investing in new innovations when they spot an opportunity."

During an executive event sponsored by Philips at Becker's 8th Annual CEO & CFO Roundtable in Chicago, many of the 20 hospital leaders engaged in a discussion about current barriers to innovation and how providers might overcome them by working to achieve true interoperability and selecting technology companies committed to their partners' success. This white paper is based on their conversation and a follow-up interview with the Philips team.

Identify the ROI

Healthcare technology companies and providers are living in two distinct financial realities. While providers' profitability levels are shrinking, the health IT market is booming as the demand for solutions to support the transition to data-driven, value-based healthcare has intensified. The global digital health market is projected to reach a valuation of more than $234 billion in 2023, up from 2019's estimated $147 billion, according to a 2019 Frost & Sullivan report. The pace of technological innovation in healthcare is rapid, but the capacity for most provider organizations to capitalize on these developments is limited. For providers, budgets are tight and the very process of identifying funds to support innovation is a painstaking one for many organizations.

"One of the complaints we hear frequently from providers is that when they're looking to buy new [technology] systems, they have to go through a traditional capital procurement cycle, which can take 12 to 18 months," Mrs. Viola said during the event. "By the time they make a decision on what to buy, the technology might be obsolete, or things may have changed at the organization that negates the reason for buying the technology in the first place."

With limited capital and windows of time to effect positive change, it's important that providers can act expeditiously when investing in innovation. But to do that, healthcare leaders need to have a pulse on their organizational needs from both a clinical and operational level.

"For me, the most critical question we need to be able to answer for our organizations is, 'What do we need?'" said the CEO of a 99-bed hospital in the Southwest. "Our philosophy is that if we think we're going to see a real return on the investment, then we'll put together the capital somehow. But with all the different systems out there, you really have to answer that question and understand if you're really going to be getting the value you think you're getting."

As payment models have shifted, so too has the definition of ROI for many providers. As the vice president of supplier performance at a $20-billion academic health system in the Northeast pointed out, "An ROI in a fee-for-service world is not an ROI in a value-based world." The supply chain leader added that both providers and vendors need to have the ability to define value-based for health system innovation efforts to succeed. Vendors need to be able to detail how their solution not only improves patient care but does so in a way that supports better reimbursement or improved efficiency.

The challenge of big data and interoperability

Once provider leaders identify a critical need that can be met by a new technology system or equipment investment, they must next confront the challenge of interoperability and big data. Disparate technology systems are prevalent in healthcare, meaning providers often don't have the luxury of being able to simply plug any solution into their current EHR system without experiencing operational mishaps. Interoperability issues can trigger a series of unintended consequences that can ultimately compromise patient safety, according to HHS' Office of the National Coordinator for Health Information Technology.

The issue of interoperability can be particularly vexing at large organizations, as the vice president of supplier performance from the academic health system in the Northeast pointed out: "I've got 290,000 medical devices that I have to manage, but I've also got a nonhomogeneous technology system, so I've also got to deal with interoperability," he said, adding that the prevalence of data in healthcare and the number of stakeholders working to commodify that data ultimately undermines interoperability. Today, health plans, technology vendors and providers are all looking for ways to use data to improve operations and patient care. These efforts sometimes result in the direct or indirect monetization of data and at large healthcare organizations can create an environment that's inhospitable to standardization and interoperability. "As much as I can standardize, I do, but too many entities are trying to monetize and all that data and that complicates things even more," the supply chain leader said.

The medical director and vice chair for systems integration at a major teaching hospital and medical research organization on the East Coast argued that true progress on interoperability won't happen without good-faith collaboration between technology companies and providers. The physician-leader suggested that vendors and providers engaged in these kinds of collaborations need a comprehensive understanding of the specific benefits interoperability can bring to an organization.

"Together, we have to ask what does this interconnectivity really mean? What is the value of this effort?" he said. "We have to take a step back and answer those questions, and then the way we integrate [technology solutions] will become more meaningful."

Without true interoperability in healthcare, many of the insights locked inside the industry's reams of data will remain invisible. The CMO of a nonprofit, six-hospital health system in the South described the current healthcare environment as rich in data, but poor in actionable information. He argued that vendors and providers have to come together to solve this issue, extract insights from data and support better patient care. "The real challenge is looking at all this data and finding out what's actually going on so we can positively impact care for patients," he said.

Find partners committed to long-term innovation and patient care

Meaningful collaboration between vendors and providers occurs when both entities come together with the goal of improving care for patients. Health system leaders should look to team up with vendor partners that have a clear understanding of their organization's financial constraints, operational and clinical challenges and are committed to supporting better patient care. Healthcare technology companies and medical device companies should be willing to share some their client's risk should the solution not yield the desired ROI. Technology companies should also be willing to make long-term commitments to support innovation at their provider partners' facilities and offer solutions that promote, not detract from, true interoperability, according to Mrs. Viola.

The Philips leader added that commitments to further technology updates and innovations over a set period of time should be built into the price when a provider purchases a new technology system or piece of sophisticated medical equipment. Not only would such an arrangement offer providers a firm commitment from vendor partners, but it would allow for greater financial transparency as leaders will have a clearer idea of just how much the investment will cost to maintain long-term. This knowledge can allow them to more effectively manage their organization from a business standpoint.

"When a provider buys a solution, they should pay for it over a set period of time with no hidden costs and no surprises," Mrs. Viola expressed to Becker's. "And the vendor needs to put skin in the game to ensure that the system is delivered and meets the providers needs over that set time period."

Today's hospital and health system leaders should look to partner with technology companies that can offer substantive, practical solutions that account for both clinical, operational and financial challenges. A certain level of adaptability should also be built into these relationships. Providers don't have the luxury of thick operating margins — the leaders of these organizations must strategize to meet both the challenges of today and anticipate those of tomorrow. Industry vendors should empower their provider partners to do just that by working side-by-side with them on shared goals and KPIs.

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